Maitri

A friend who tells the family what to do now

When the bleeding starts, a family has minutes. Maitri gives them the next step, and shows why.





Team Maitri | Aafreen Khan, Aastha Pandya

recHERsion 2026 | Problem Statement 1: Women's Healthcare

Contact is high. The decision is late.

25.2M

births a year in India

Secondary source

48.6%

of maternal deaths in one Indian hospital study involved a delay in deciding or seeking care

Verified, single study

1 in 3

women still have fewer than four antenatal visits

Secondary, derived

Why today's options fall short

A card has one level

No action plan, no voice

Chatbots need typing and signal

Hidden logic, may say "fine"

Helplines need a phone and a clear head

The decision is still the family's

Sources and confidence labels: docs/PITCH_FACTS.md

Who uses it first



The woman

Pregnant or within 42 days of birth



ASHA or ANM

Checks one woman after another



The family

Decides if she goes now

About 36% of women do not own a phone (secondary), so ASHA and family modes come first.

First users: ASHAs and families in a Hindi-speaking district with patchy signal. Only TAM is a sourced number. SAM and SOM are labelled "to be sized" on purpose.

Size of the opportunity

TAM

25.2M births a year in India

Secondary source

SAM

Births in Hindi-first, ASHA-served states

To be sized from SRS data

SOM

One district pilot, ASHAs of one block

To be sized with the health mission

Say it. See why. Know what to do.

An offline, Hindi-first checker that turns danger signs into one clear next step for the family.

Voice or tap

Speak it, or tap Yes, No, Not sure

Shows its rule

Every verdict names its sign and source

One RED sign, one call

The check stops and the call button shows

Slip for the ASHA

Share with consent. No name or number

Works offline

After one visit, in airplane mode

Private by default

No account needed. Exit now clears all

Prototype walkthrough



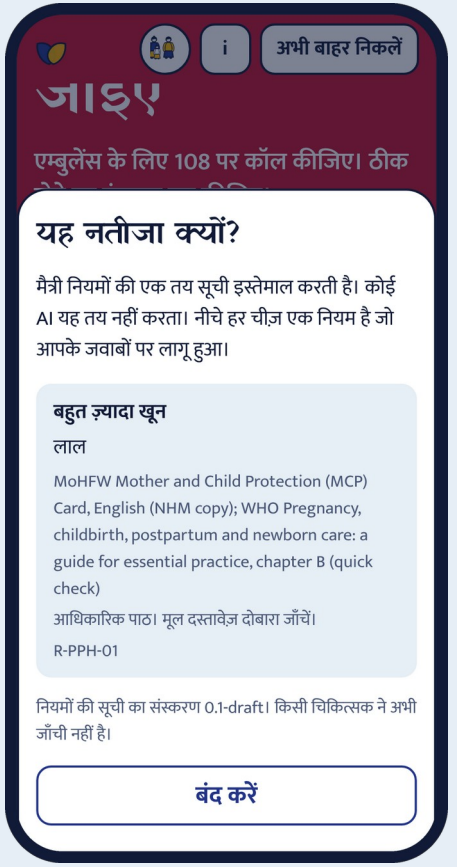
Emergency call first



Say what is happening



One RED sign, one call



Why this result

Scan for code, demo video and the live app link

github.com/aafreenzy/maitri

Demo video: docs/demo

How it is built

Voice path: no model decides

Browser speech → **Transcript she can edit** → **Word lists (Hindi, English)** → **Same Yes / No / Not sure** → **Rule engine**

On the phone

- React 18 + TypeScript + Vite
- Rules in one JSON file, 46 cases
- Service worker: offline
- Hindi and English strings
- Browser speech recognition

Checks

- 205 automated tests
- Python re-check, 12,420 cases
- Database tests in real Postgres
- axe audit, small-screen audit
- CI on every pull request

Optional cloud (Supabase)

Sign in with Google or phone code

checkins

user_id, entry_date, phase, answers (json), tier, rules_version

profiles

user_id, stage, due_date, delivery_date, language

Row level security: each woman reads only her own rows. Tested in Postgres.

Built, not connected yet

Feasible now. Clear next steps.

Static site

No server for the check

Offline

Verified with the network cut

Low cost

About INR 6 per language for voice clips

Privacy first

No account needed

Now

Clinician review. ASHA field test. Real-phone voice test.

Next

Odia, Assamese, Bengali. Connect sign-in and the daily log.

Then

Visit and vaccine schedules. Scheme help.

Later

Facility finder from a verified dataset.

Free for her. Funded by those who run care.



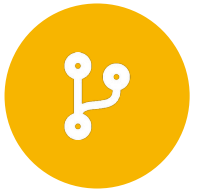
State missions and NGOs

Licence, localisation, ASHA training and a district dashboard (with consent)



CSR and grants

Fund new languages, voice clips and clinician review



Open core

Rules and app stay open and auditable. Services are paid.

We never sell data. No ads. Women and families pay nothing. This is a proposed model and has not been tested with a funder yet.

What broke. What we changed.

The build failed on main

A new screen changed the flow and a test was not updated. Tests now gate the build and CI runs on every pull request.

Buttons clipped on short phones

Items were squeezed in a scrolling column. One layout rule and a 15-screen audit fixed it.

A misheard "No" could hide a sign

A spoken No is confirmed. A misheard Yes only over-escalates, so it goes straight through.

Voice cannot be fully tested here

Tests use a simulated recogniser, and we say so. A real-phone test is the next step.

Trade-offs we chose

Rules, not AI | **Over-escalate, never under** | **Offline over cloud** | **No numeric thresholds**

Sources and credits

- MoHFW Mother and Child Protection card | HBNC leaflet | WHO essential practice guide, chapters B and D
- JSSK guidelines (NHM) | PIB note on Tele-MANAS 14416
- NFHS-6 findings and SRS maternal mortality bulletin 2021-23 (secondary coverage, labelled in the repository)
- BMC Pregnancy and Childbirth 2019 delay study | Cochrane 2020 mobile messaging review | Kenya PROMPTS trial
- Built with React, TypeScript, Vite, Vitest, PGlite (tests), Supabase client. Fonts: Yatra One and Mukta (SIL OFL). Pictograms drawn for this project.
- AI assistance: built with Claude Code. The team is responsible for all content.

github.com/aafreenzy/maitri

Education tool. Does not diagnose. Rules are a draft, not reviewed by a doctor.